

The same device, two documentation strategies

Case study: a Class IIa digital clinical thermometer. Left — the uniform-rigor file that industry practice submits (every document, every class, because nobody dares to cut). Right — the minimal compliant set DocLite derives, with a clause citation for every document removed. All figures computed live by the deterministic engine.

Class IIa

Digital Clinical Thermometer

EU MDR Annex VIII · Rule 10 (active diagnosis/monitoring) · Notified Body required

THE UNIFORM SUBMISSION – INDUSTRY PRACTICE

52 documents · ~1,878 h

The full corporate template set, applied identically from stethoscope to implant. Every document drafted, reviewed, approved, maintained on every design change, and defensible at every audit — including the ones the law never asked for.

DOCLITE – THE PROVABLE MINIMUM

35 required · 962 h

- +9 conditional (188 h) – each with the exact determination to document
- 8 eliminated (728 h) – each with the clause proving it
- ~822 hours (44%) saved — and, more importantly, every omission pre-answered for the auditor.

THE EXCESS, ITEMIZED – WHAT THE UNIFORM FILE CARRIES FOR NOTHING

DOCUMENT IN THE UNIFORM SUBMISSION	HOURS	WHY IT WAS NEVER REQUIRED	CITATION
Clinical Investigation (pre-market study)	400 h	The Art. 61(4) obligation applies only to Class III and implantables — clinical evidence comes from the right-sized clinical evaluation instead	Art. 62 + Annex
Software Lifecycle File (IEC 62304)	100 h	No software in the device — the entire lifecycle set is inapplicable	Annex I §17.2
Sterilization Validation	60 h	Not supplied sterile	Annex I §11.4
Cybersecurity Documentation	60 h	No software or connectivity	Annex I §17.4

Packaging & Sterile-Barrier Validation (ISO 11607)	48 h	No sterile barrier system to validate	Annex I §11.4
Summary of Safety & Clinical Performance (SSCP)	40 h	SSCP applies only to implantables and Class III	Art. 32
PMS Report (Class I form)	12 h	Class IIa produces a PSUR under Art. 86 instead — the Art. 85 report is the Class I instrument	Art. 85 vs 86
Implant Card & Patient Information	8 h	Not an implantable device	Art. 18
Total excess in the uniform submission	728 h	— plus its lifetime of reviews, updates and audit exposure on every design change	

— GROUND TRUTH — A REAL CLEARED SUBMISSION AGREES

EVIDENCE LAYER	FDA 510(K) K163603 — REAL CLEARED DIGITAL THERMOMETER (2017)	DOCLITE'S VERDICT FOR THE SAME DEVICE	MAT
Biocompatibility	Submitted: ISO 10993-5, ISO 10993-10 (probe contacts mucous membrane)	Required — body-orifice contact triggers ISO 10993-1 evaluation	✓
Electrical safety & EMC	Submitted: IEC 60601-1, IEC 60601-1-2, IEC 60601-1-11 (home use)	Required — active (powered) device	✓
Clinical accuracy performance	Submitted: ASTM E1112, ISO 80601-2-56	Required — measuring function verification (design verification)	✓
Clinical investigation (human study)	Absent — cleared via predicate equivalence, no clinical study	Eliminated — Art. 61(4) applies to III/implantables only	✓
Sterilization validation	Absent — device not sterile	Eliminated — Annex I §11.4 not applicable	✓
Software lifecycle file (IEC 62304)	Absent — no software documentation listed	Eliminated — no software in device	✓
Cybersecurity documentation	Absent — no connectivity	Eliminated — Annex I §17.4 not applicable	✓

The regulator cleared this real thermometer with exactly the testing DocLite marks Required — and **none of the documents DocLite eliminates appear in the cleared file**. Source: [FDA K163603 510\(k\) summary, accessdata.fda.gov](#) (public record).

Honesty notes: (1) K163603 is a US 510(k) — its public summary covers the evidence/testing layer, not the full QMS file (FDA inspects QMS separately), so the validation applies to the Product Verification and Clinical scoping specifically. (2) Real EU technical files are confidential, so the EU-side comparison uses the representative uniform-rigor file — the full 52-document regulatory superset — as the ceiling, not a named company's submission. Full-file validation path: pilot back-testing with partner RA teams.

